

Pihlajalinna Oyj

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COMPANY PROFILE

Pihlajalinna is a Finnish provider of private medical, diagnostic, dental and occupational-health services and outsourced health and social-care operations for public authorities. Its economics combine appointment-led private demand, recurring employer contracts and long-duration public-sector agreements. The group operates through Healthcare Services and Outsourcing Services reporting segments, supported by shared clinical, digital and administrative infrastructure.

RECOMMENDATION

HOLD

12-month horizon

CURRENT PRICE

10.50 EUR

as of report date

TARGET PRICE

10.50 EUR

12-month fundamental

IMPLIED UPSIDE

0.0%

vs. current price

MARKET CAP

EUR 236 m

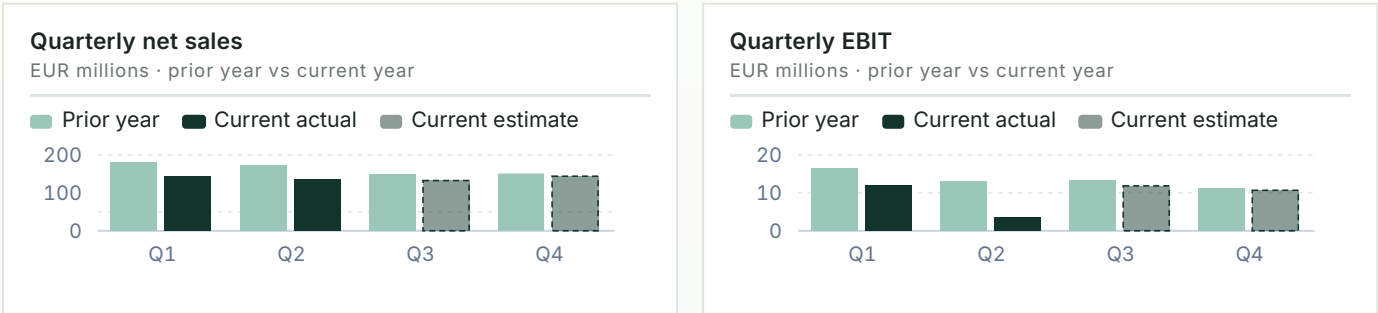
shares × price

ENTERPRISE VALUE

EUR 512 m

mcap + EV adjustments

P/E 2026E	EV/EBITDA 2026E	FCF YIELD 2026E	DIVIDEND YIELD 2026E	NET DEBT / EBITDA 2026E
12.2×	5.8×	6.8%	1.3%	3.1×



RESEARCH SNAPSHOT

The call, the math, and the three reasons why

02

A one-page summary of the recommendation and the evidence behind it; the following pages provide the detail.

RECOMMENDATION HOLD 12-month horizon	TARGET PRICE 10.50 EUR 12-month fundamental	CURRENT PRICE 10.50 EUR as of report date	IMPLIED UPSIDE 0.0% vs. current price
MARKET CAP EUR 236 m shares × price	ENTERPRISE VALUE EUR 512 m mcap + EV adjustments	P/E · EV/EBITDA 2026E 12.2x · 5.8x history + peers, see p. 18–19	FCF · DIV YIELD 2026E 6.8% · 1.3%



01 Healthcare carries the franchise

Private Medical Services and Occupational Healthcare represent about three-quarters of allocated enterprise value. Their recurring clinic infrastructure, employer contracts and ability to redirect patients across diagnostics and specialists provide the clearest route to earnings durability after public-outsourcing expiries.

75% of EV

02 The outsourcing reset is identifiable

The expected 2026 revenue decline is concentrated in known contract expiries and divestments rather than an unexplained collapse in group demand. Northern Pirkanmaa contributes a minimum EUR 222 million contract through at least July 2031, partly rebuilding the book on more visible terms.

EUR 222m contract

03 Leverage preserves both upside and risk

Net debt of EUR 273 million exceeds the EUR 236 million equity value. Deleveraging can therefore create meaningful per-share value, but weak working-capital conversion or refinancing pressure can remove it just as quickly.

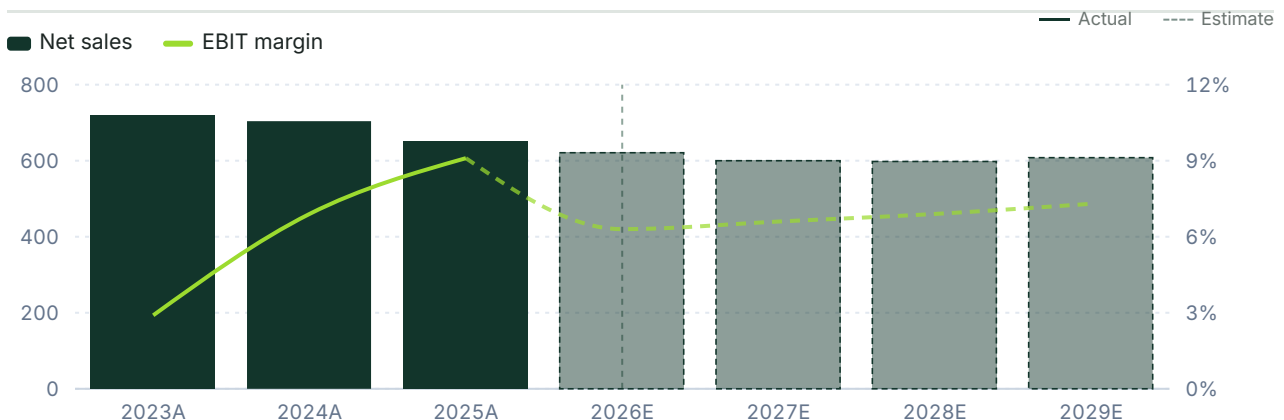
3.09x 2026E

THESIS BREAKER

The thesis breaks if 2027 EBITDA remains below EUR 75 million and net debt does not decline from EUR 273 million.

Net sales and EBIT margin

EUR millions; EBIT margin on right axis



SHARE PRICE DEVELOPMENT

Price trajectory and valuation anchors

03

Weekly closing prices over 36 months with the 12-month target price, alongside key market data.



CURRENT PRICE 10.50 EUR 21 August 2026	TARGET PRICE 10.50 EUR 12-month	IMPLIED UPSIDE 0.0% vs. current	52-WEEK HIGH 17.30 EUR past 12m
52-WEEK LOW 9.61 EUR past 12m	1-YEAR CHANGE -36.9% price only	3-YEAR CHANGE +26.3% price only	REPORT DATE 21 August 2026

READING THE MOVE

the evidence supports three competing forces—strong 2025 profitability, the known loss of large outsourcing agreements, and the July 2026 reduction in revenue and margin guidance. H1 Healthcare Services profitability improved, but working-capital absorption and contract transitions limited the quality of the earnings signal. No more precise attribution of the share-price path is established in the supplied evidence. The current price is aligned with the EUR 10.50 target.

VALUE BREAKDOWN · PART 1

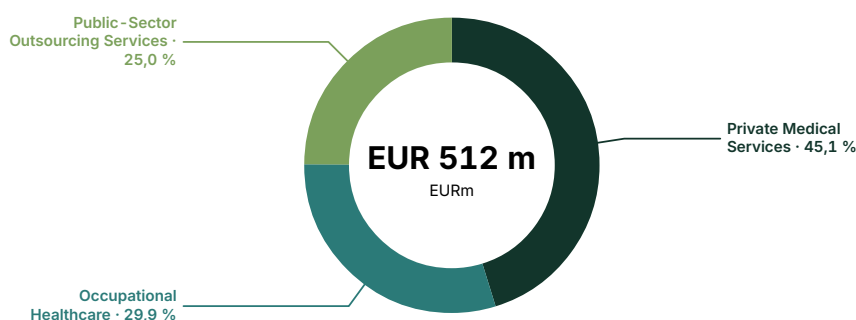
Enterprise - value allocation by business division

04

Where the market value sits across current businesses and long-term strategic options.

Enterprise value by business division

EUR millions · current analytical allocation



VALUE AND EARNINGS BY BUSINESS DIVISION

Analytical enterprise-value allocation against reported economics. "Reported" rows match a verified company-reported segment; "Modelled allocation" rows are model-side splits. Business divisions may differ from company-reported accounting segments.

	EV	EV %	REVENUE	REV %	COMP. EBIT	COMP. EBIT % OF TOTAL	BASIS
Private Medical Services	231	45.1%	254	39.0%	21	35.6%	Modelled allocation
Occupational Healthcare	153	29.9%	185	28.3%	8	13.6%	Modelled allocation
Public-Sector Outsourcing Services	128	25.0%	199	30.5%	27	45.3%	Modelled allocation
Total	512	100.0%	652	97.8%	59	94.4%	

VALUE BREAKDOWN · PART 2

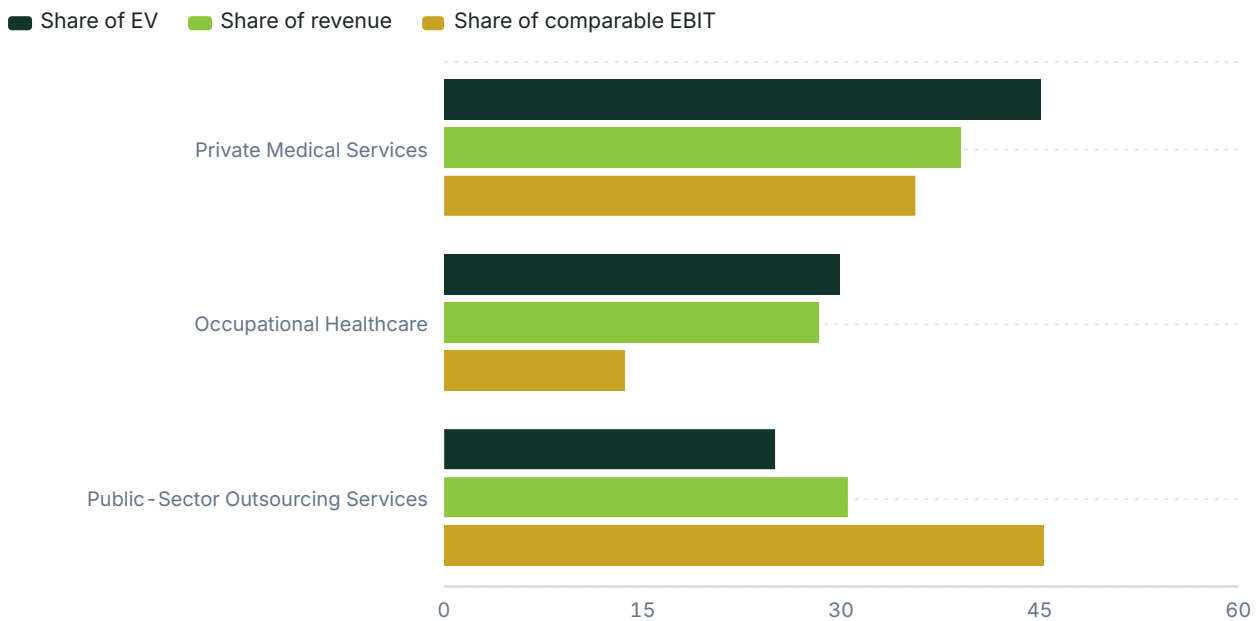
Where value sits versus where money is earned

05

Each business division's share of enterprise value against its share of group revenue and profit. A division holding value it does not yet earn is where the valuation rests on the future; the full figures are in the allocation table on the previous page.

Share of enterprise value vs revenue and profit

Per cent of group total · modelled allocation



INTERPRETATION

The locked EUR 512 million enterprise value is allocated EUR 231 million to Private Medical Services, EUR 153 million to Occupational Healthcare and EUR 128 million to Public-Sector Outsourcing Services. The allocation shows that three-quarters of value depends on Healthcare Services economics, even though outsourcing contributed a large share of FY2025 EBIT. Corporate functions are reflected through group cash flows and the locked bridge rather than presented as a fourth business division.

INVESTMENT CASE

The call and how the business creates value

06/07

Why the valuation and operating evidence point to the recommendation, and the map for the business division analysis that follows.

RECOMMENDATION CASE

HOLD. The 12-month target price is EUR 10.5, implying 0.0% upside from the current price of EUR 10.50.

The current price assumes that the 2026 revenue reset is largely contractual rather than evidence of permanent deterioration. The followed model forecasts EUR 621 million of 2026 revenue and EUR 88 million of EBITDA, but the company's 22 July guidance is only EUR 550–570 million and a 9–10% adjusted EBITA margin. That conflict matters: the model definition and management guidance are not identical, yet the model's revenue is plainly above the latest reported range. Our valuation consequently relies on 2027 divisional EBITDA rather than treating the 2026 model as an uncontested operating forecast.

The assembled evidence supports a mixed conclusion. Private and occupational healthcare retain structural demand from public-system waiting times, care deficits and employer concern over absence costs, while occupational coverage increased to 284,000 individuals by H1 2026. Against that, Kela reimbursement for ordinary private doctor appointments fell from EUR 30 to about EUR 8, selected occupational services became VAT-taxable, and private-healthcare wages rise from September 2026. In outsourcing, the Northern Pirkanmaa award restores a durable revenue stream, but expiries and divestments still remove about EUR 85 million of 2026 revenue.

The strongest argument for BUY is that the equity is a leveraged claim on modest operating improvement: each EUR 10 million of sustainable EV improvement equals about EUR 0.44 per share before any debt reduction. Healthcare Services profitability strengthened in H1, net debt is forecast to fall after 2026, and a 2027 EBITDA multiple near direct-peer levels could support appreciable upside. That does not change the call because guidance risk, receivables absorption and a 2028 refinancing deadline make the path to that upside less dependable than the endpoint.

The call turns positive if revenue enters 2027 near EUR 600 million, EBITDA remains at least EUR 87 million, and net debt falls toward EUR 249 million without acquisition spending. It turns negative if occupational contract repricing fails to cover wage and VAT effects, or if outsourcing exits push EBITDA below EUR 75 million while net debt remains above EUR 270 million. Those are observable operating conditions, not share-price thresholds.

ECONOMIC MAP

Enterprise value forms first in Private Medical Services, where EUR 35 million of modelled 2027 EBITDA at 6.6x produces EUR 231 million. The one number that matters is realised revenue per clinician slot after the 2026 Kela reimbursement cut and wage increase. Stable volume with price and mix covering labour inflation supports the valuation; declining self-pay volume would expose the fixed clinic base.

Occupational Healthcare contributes EUR 153 million from EUR 20.4 million of modelled EBITDA at 7.5x. Its decisive number is revenue per covered individual: the report assumes EUR 650 across 284,000 people. Public-Sector Outsourcing contributes EUR 128 million from EUR 32 million of EBITDA at 4.0x, with cash conversion and contract retention more important than reported margin.

The EUR 512 million parts sum exactly equals the locked enterprise value. Deducting EUR 273 million of net debt and EUR 3 million of other claims leaves EUR 236 million of equity. The stock is therefore a leveraged portfolio of one discretionary clinical business, one recurring employer business and one concentrated public-contract business.

ANALYTICAL ANCHOR

The target is earned only if 2027 divisional EBITDA reaches EUR 87.4 million in aggregate and the post-transition balance sheet begins to deleverage.

08.1

Private Medical Services

Business division deep dive — Profit engine.

THE PRIZE IS FINLAND'S PRIVATELY PROVIDED CARE SPEND

Private Medical Services competes for the portion of Finnish healthcare expenditure that patients, insurers, employers and public purchasers are willing to redirect to private appointments, diagnostics and procedures. Pihlajalinna estimated its core addressable Finnish market at EUR 3.5 billion on 22 July 2026 and expected 3–4% annual growth to 2030, while Terveystalo used a broader EUR 4.5 billion definition in July 2026. The difference is scope, not necessarily a disagreement about observed transactions: the larger figure includes more publicly funded private provision and adjacent services. This valuation uses EUR 3.5 billion as the reachable market because it is the narrower company-defined opportunity, with EUR 4.5 billion retained as an upper boundary. Against total Finnish health and social-welfare operating expenditure of EUR 24.6 billion in 2024, even the broader private market remains a minority channel, leaving structural room for selective outsourcing and self-pay growth without assuming wholesale privatisation.

The immediate consequence for Pihlajalinna is more modest than the headline market. The report models FY2025 Private Medical Services revenue at EUR 254.2 million: restated Healthcare Services external revenue of EUR 438.8 million less modelled Occupational Healthcare revenue of EUR 184.6 million. The latter equals 284,000 covered individuals multiplied by an assumed EUR 650 annual revenue per covered person; because the company does not disclose occupational revenue separately, both resulting division figures are estimates rather than reported segments. EUR 254.2 million represents 7.3% of the EUR 3.5 billion market, although the numerator excludes occupational care while the denominator includes it, so this is an orientation ratio rather than a claimed market share. The valuation does not require a dramatic share gain: it requires the private-medical book to preserve roughly its current scale while raising utilisation and mix sufficiently to produce EUR 35 million of 2027 EBITDA.

PRIVATE MEDICAL SERVICES
FACT SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
EUR 254m

MODEL ESTIMATE COMPARABLE EBIT
EUR 21m

MODELLED FY2025 REVENUE
EUR 254.2m

MODELLED EBIT MARGIN
8.3%

2027 VALUATION METRIC
EUR 35.0m EBITDA

SELECTED EV/EBITDA
6.6x

RELEVANT FINNISH MARKET
EUR 3.5–4.5bn

TERVEYSTALO REVENUE PER VISIT
EUR 177.64 (2025)

ORDINARY KELA REIMBURSEMENT
About EUR 8/visit (2026)

REQUIRED OPERATING CONDITION
Pricing above wage inflation

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.1

Private Medical Services (continued)

Business division deep dive — Profit engine.

VOLUME, PRICE AND MIX DETERMINE WHETHER THE MARKET BECOMES EARNINGS

Pihlajalinna does not disclose a full-year private-medical visit count, clinic capacity utilisation or revenue by procedure. The most useful external unit benchmark is Terveystalo's 7.2 million 2025 visits and EUR 1.3 billion revenue, equivalent to EUR 177.64 per visit, although that calculation mixes occupational, diagnostic and other revenue with visits and is not a pure appointment price. Applying EUR 150–190 of revenue per equivalent visit to the modelled EUR 254.2 million private-medical revenue implies approximately 1.34–1.69 million equivalent visits. This report uses 1.50 million as an operating reference, not as a reported volume. At that volume, each EUR 5 change in realised revenue per equivalent visit changes annual revenue by EUR 7.5 million before volume response.

The economic leverage comes from clinician utilisation rather than simply charging more for a consultation. Fixed clinic leases, digital infrastructure, scheduling and administration are already present; filling an unused specialist slot should therefore have a higher contribution margin than opening a new location. Diagnostics and surgery can increase revenue per patient episode, while dental and physiotherapy broaden the referral pathway. Pihlajalinna worked with about 2,300 private practitioners and employed about 4,500 people in late 2025, giving it a flexible but not costless capacity base. Practitioner compensation frequently varies with activity, reducing downside operating leverage, whereas employed clinical and support labour creates greater sensitivity to utilisation.

- Pihlajalinna: modelled EUR 254.2m private-medical revenue and roughly 1.34–1.69m equivalent visits in FY2025.
- Terveystalo: EUR 1.3 billion revenue, 7.2m visits and about 25–30% of the Finnish privately provided market in 2025.
- Mehiläinen: EUR 2.4 billion group revenue in 2025, but international operations and undisclosed Finnish private-medical mix prevent a direct share comparison.
- Aava and Pikkujätti: third-party estimates range from EUR 140.1m to EUR 147.0m for 2025; the EUR 140.1m euro-denominated estimate is weighted more heavily because the EUR 147.0m entry was labelled in US dollars.

REIMBURSEMENT CUTS TEST DEMAND ELASTICITY

Ordinary private general-practitioner and specialist reimbursement fell from EUR 30 to about EUR 8 in 2026, while remote-consultation reimbursement also fell to EUR 8. For a patient, that raises the net cost by roughly EUR 22 if the provider's gross price is unchanged. The effect is meaningful for price-sensitive self-pay primary care but smaller relative to the total price of many specialist, diagnostic and surgical episodes. Gynaecology and psychiatry retained higher reimbursements of EUR 70 and EUR 50–60 respectively, and physiotherapy regained a EUR 15 reimbursement for up to four annual sessions from May 2025. These exceptions make service mix important: a uniform volume shock across all specialties would overstate the exposure.

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.1

Private Medical Services (continued)

Business division deep dive — Profit engine.

Public waiting times remain the counterweight. A patient choosing between a delayed public appointment and immediate private treatment values speed, continuity and access to diagnostics, not just the reimbursement amount. The 65- and -over freedom-of-choice pilot caps the patient's private GP out-of-pocket fee at the public health-centre rate of EUR 30.20, creating a targeted volume channel even as general reimbursements fall. Service vouchers provide another route; Western Uusimaa set an adult GP voucher value of EUR 55 for 2026, but county-level terms are fragmented and cannot be generalised across Finland. The base case therefore assumes flat to slightly lower self-pay primary-care volume, offset by occupational referrals, insured patients, selected reimbursement-supported categories and improved procedure mix.

LABOUR INFLATION CAN ABSORB THE UTILISATION GAIN

The private-healthcare collective agreement sets a 2.22% general wage increase from September 2026, an additional 0.7% for selected demand-group employees and a EUR 183 one-time payment in December. A further 2.0% general increase and 0.4% local installment apply from August 2027. Reported Healthcare Services employee-benefit expense was 37.2% of revenue in Q4 2025, materially below Outsourcing Services because private practitioners and purchased services sit outside employee costs. As a first-order estimate, a 2.22% wage increase on a 37.2% personnel-cost ratio adds about 0.83% of revenue to cost before productivity or price action. On EUR 254.2 million, that is roughly EUR 2.1 million if the ratio applies to this division.

This sensitivity is manageable but not trivial relative to modelled EBIT of EUR 21 million. A 1% price increase across the book produces EUR 2.5 million of revenue before elasticity, approximately matching the first-order wage burden. The real question is whether pricing can be implemented without losing price-sensitive patients after the reimbursement cut. Lower national vacancies—31,400 in June 2026 versus a much higher 2022 peak—suggest recruitment pressure may be easing, but specialist availability can remain locally constrained even when aggregate labour markets soften. The valuation assumes price and mix cover wage inflation over a full year and utilisation adds only a modest incremental benefit.

COMPETITIVE POSITION IS CREDIBLE BUT NOT DOMINANT

Terveystalo is the closest listed comparator and claims 25–30% of the privately provided Finnish healthcare market. Mehiläinen reports much greater total group revenue, but its international expansion and broader service portfolio make group size an unreliable measure of the specific Finnish private-medical contest. Pihlajalinna's advantage is a national brand, integrated occupational referral base and ability to combine appointments, diagnostics and procedures. Its disadvantage is lower scale than the two largest operators, limiting procurement leverage, marketing reach and the density of specialist availability in every locality. Smaller providers such as Aava can still compete through local reputation, specialist concentration and customer experience.

08.1

Private Medical Services (continued)

Business division deep dive — Profit engine.

Market concentration creates both protection and political risk. The three large chains possess scheduling technology, occupational contracts and clinic networks that are difficult to replicate quickly, but visible private-care price increases can attract regulatory and purchaser scrutiny. No evidence supplied establishes a direct Pihlajalinna market-share percentage for private medical services, so the report does not infer one from group revenue. The operating test is instead observable: stable Healthcare Services revenue after adjusting for portfolio changes, maintained clinician availability, and profitability that does not depend solely on cost reductions.

VALUATION RESTS ON EUR 35 MILLION OF 2027 EBITDA

The SOTP assigns Private Medical Services EUR 231 million of enterprise value, calculated as modelled 2027 EBITDA of EUR 35 million multiplied by 6.6x. The multiple sits above Terveystalo's 5.2x 2027E EV/EBITDA because the pool excludes comprehensive outsourcing risk, but below Pihlajalinna's normalised historical 8.14x and well below growth-healthcare outliers. The EBITDA estimate corresponds to about 13–14% of modelled revenue if revenue remains near EUR 255–265 million. That is demanding relative to the division's modelled EBIT margin, but depreciation and amortisation bridge a meaningful part of the difference and clinic utilisation can support incremental margins.

The value breaks if realised revenue per equivalent visit falls, if volumes decline materially after reimbursement changes, or if wage inflation cannot be passed through. At EUR 30 million of EBITDA and the same 6.6x multiple, division EV falls to EUR 198 million, removing EUR 33 million or about EUR 1.47 per share from equity value before any offsetting debt reduction. At EUR 40 million, EV rises to EUR 264 million, adding the same amount. This symmetrical sensitivity shows why utilisation and pricing—not the broad market growth headline—are the decisive variables.

PRIVATE MEDICAL SCALE AND POSITIONING, LATEST AVAILABLE FIGURES

OPERATOR	SCALE / VOLUME	MARKET POSITION	KEY LIMITATION
Pihlajalinna private medical	Est. EUR 254.2m (2025)	Est. 1.34–1.69m visits	No disclosed visit count
Terveystalo	7.2m visits (2025)	25–30% broad market	Broader revenue mix
Mehiläinen	EUR 2,413.2m group (2025)	Largest by group revenue	Includes international sales
Aava and Pikkujätö	EUR 140.1m est. (2025)	Material national challenger	Third-party estimate
Fimlab	EUR 174.0m est. (2025)	Diagnostics specialist	Not full-service comparable

08.2

Occupational Healthcare

Business division deep dive — Profit engine.

THE PRIZE IS THE EMPLOYER COST OF ILLNESS, NOT MERELY CLINIC BILLING

Occupational Healthcare ultimately competes for employers' willingness to prevent absence and preserve productivity. Finnish sickness-related absence and lost productivity displace an estimated EUR 4.8–5.0 billion of annual economic value, according to Helsinki Times citing employer research in June 2026. That displaced cost is far larger than providers' occupational-health revenue and is the correct economic prize because an employer buys faster return to work, prevention and work-ability management rather than appointments alone. The labelled service market sits inside the EUR 3.5–4.5 billion privately provided healthcare market and covers approximately 2.1 million individuals. The valuation does not assume providers capture the productivity pool; it uses that pool only to explain why customers may accept price increases when measurable absence savings exceed contract cost.

Pihlajalinna reported 284,000 occupational-health individuals at H1 2026, up from about 260,000 in earlier evidence. Dividing 284,000 by Finland's 2.1 million covered population gives this report's estimate of a 13.5% individual-coverage share. That is higher than the approximately 10% historical estimate based on the older 260,000 count, and it remains well below Terveystalo's 722,000 people, or 34.4%. The immediate capture requirement is therefore retention rather than a heroic market-share target. Base valuation assumes Pihlajalinna preserves approximately 280,000–290,000 covered individuals while increasing revenue per member enough to offset labour and VAT effects.

REVENUE IS RECONSTRUCTED FROM MEMBERS BECAUSE THE COMPANY DOES NOT SPLIT IT

The company books Occupational Healthcare within Healthcare Services and does not report separate revenue. This report models FY2025-equivalent revenue at EUR 184.6 million, calculated as 284,000 covered individuals multiplied by assumed annual revenue per member of EUR 650. The assumption is intentionally presented as a range-sensitive estimate: EUR 500 per member would produce EUR 142 million, while EUR 800 would produce EUR 227 million. The selected midpoint reflects a blended contract containing preventive services, medical care, diagnostics and specialist referrals rather than a basic statutory subscription. It is not derived from the advertised EUR 8 monthly entry package, which represents a minimal small-company offer and would omit activity-based care.

OCCUPATIONAL HEALTHCARE
FACT SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
EUR 185m

MODEL ESTIMATE COMPARABLE EBIT
EUR 8m

COVERED INDIVIDUALS
284,000 (H1 2026)

MODELLED ANNUAL REVENUE
EUR 184.6m

ASSUMED REVENUE PER MEMBER
EUR 650/year

SHARE OF COVERED POPULATION
About 13.5%

2027 VALUATION METRIC
EUR 20.4m EBITDA

SELECTED EV/EBITDA
7.5x

DOCTOR LIST PRICE
EUR 255.90/hour (2025)

MAIN CONTRACT TEST
Indexation versus wages

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.2

Occupational Healthcare (continued)

Business division deep dive — Profit engine.

The resulting arithmetic leaves EUR 254.2 million of restated Healthcare Services external revenue for Private Medical Services. This is a modelling allocation, not reported segment information, and the two division estimates should be read together. An alternative occupational ARPU changes the division split but not the verified Healthcare Services total. It would, however, change the SOTP if different multiples are applied to recurring occupational revenue and more discretionary private-medical revenue. The base case uses the midpoint because recurring contracts merit a higher multiple, but not so high an ARPU that value is mechanically shifted into the higher-rated pool.

CONTRACT RETENTION AND SERVICE INTENSITY ARE THE OPERATING ENGINE

Occupational-health contracts normally combine a recurring preventive component with usage-based medical services. The EUR 8 per employee per month small-company package demonstrates that the statutory floor can be inexpensive, while Pihlajalinna's published occupational-health doctor rate of EUR 255.90 per hour illustrates the much higher value of activity-based clinical delivery. At EUR 650 annual modelled revenue per covered individual, the average monthly amount is EUR 54.17. That could represent a modest fixed fee plus approximately two to three hours of mixed nurse, physician, digital and diagnostic activity per year, depending on discounts and service mix. Because exact internal utilisation and cost per member are undisclosed, the report does not claim a precise visit frequency.

Retention has two layers. Human-resources buyers evaluate total contract cost, employee access and reporting, while individual users experience appointment availability and continuity. A provider can grow covered lives yet dilute economics if wins require aggressive pricing or if newly won populations consume more medical services than expected. Conversely, digital triage and preventive intervention can lower unnecessary physician use while preserving customer value. The evidence of growth to 284,000 covered individuals supports competitive relevance, but does not establish that cohort profitability improved.

- Pihlajalinna: 284,000 covered individuals at H1 2026, implying about 13.5% of Finland's 2.1m covered population.
- Terveystalo: 722,000 people through more than 25,000 corporate customers in 2025, equivalent to 34.4% coverage share.
- Mehiläinen: ranked the most recommended occupational-health provider among decision-makers in 2025, but no precise covered-life share was supplied.
- Aava: EUR 140.1m estimated total Finnish healthcare revenue in 2025; occupational-health revenue and membership were not separately available.

VAT RECLASSIFICATION CREATES A LIVE REPRICING TEST

Following a Finnish Tax Administration audit, certain occupational-health services were reclassified as VAT-taxable and Pihlajalinna updated pricing from 1 May 2026. The first-order question is who absorbs the tax: Pihlajalinna, employers, or employees through narrower service usage. If contract language permits pass-through, accounting revenue may rise with little underlying margin impact, although customer budgets can still constrain volume. If contracts are fixed or repricing is delayed, the provider absorbs part of the burden. The company has not disclosed the affected revenue base, preventing a precise EBIT sensitivity.

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.2

Occupational Healthcare (continued)

Business division deep dive — Profit engine.

A practical range can still be framed. If 10–20% of modelled EUR 184.6 million revenue is affected and the effective unrecovered burden equals 5–10% of that slice, annual EBITDA exposure would be roughly EUR 0.9–3.7 million. The base case assumes pricing and contract renewals recover most of the impact over 12–18 months, leaving less than EUR 1.5 million of recurring leakage. This assumption is critical because a EUR 2 million EBITDA miss capitalised at 7.5x removes EUR 15 million of EV, or approximately EUR 0.67 per share. Evidence of customer churn after May pricing changes would invalidate the benign reading.

WAGE INFLATION MUST BE COVERED CONTRACT BY CONTRACT

The private-healthcare agreement raises general pay scales by 2.22% from September 2026, with further targeted and one-off payments, followed by 2.4% combined general and local increases from August 2027. Occupational Healthcare shares the 37.2% Healthcare Services employee-benefit ratio reported for Q4 2025, although the true division ratio may differ. Applying that ratio to EUR 184.6 million suggests EUR 68.7 million of employee expense; a 2.22% increase would add about EUR 1.5 million on a full-year basis. The additional 0.7% applies only to selected groups and should not be applied to the entire wage base.

Contract indexation is therefore the decisive margin variable. A 1% price increase on the full revenue base adds EUR 1.85 million before elasticity, broadly enough to cover the first-order general wage rise. Yet price timing matters: September wage increases affect four months of 2026, while annual customer repricing may occur at different renewal dates. The base case assumes a temporary margin squeeze followed by recovery in 2027. A structurally fixed-price book would deserve a lower multiple than the selected 7.5x.

COMPETITION FAVOURS DENSE NETWORKS AND DATA CAPABILITY

Terveystalo has the largest explicitly disclosed occupational population at 722,000 people and benefits from nationwide clinic density. Mehiläinen's customer recommendation evidence suggests strong brand and account-management quality, while its acquisition of Heltti's occupational business effective September 2026 adds competitive intensity after this report date. Pihlajalinna remains a credible third national chain, with the advantage that its occupational base can feed private specialists and diagnostics. That internal referral pathway is economically important but should not be double counted: revenue is assigned to the service ultimately delivered, while the occupational relationship earns a higher valuation multiple for customer ownership.

08.2

Occupational Healthcare (continued)

Business division deep dive — Profit engine.

Finland's occupational-health regulation is being revised with changes effective from 1 January 2027, and occupational-safety supervisory tasks transferred to the Finnish Supervisory Agency from January 2026. The supplied evidence does not identify a permission barrier preventing Pihlajalinna from operating; the regulatory risk is instead altered service obligations, reimbursement or documentation burden. Employers remain legally engaged in arranging occupational health, which supports recurring demand. The next test is whether regulatory change increases administrative cost faster than contract pricing.

VALUATION PAYS FOR RECURRENCE, NOT AGGRESSIVE MEMBERSHIP GROWTH

The SOTP assigns Occupational Healthcare EUR 153 million of EV, calculated as EUR 20.4 million of modelled 2027 EBITDA multiplied by 7.5x. The multiple exceeds Terveystalo's group 5.2x 2027E EV/EBITDA because employer contracts are recurring and the pool excludes lower-quality outsourcing exposure, but it remains below Pihlajalinna's 8.14x normalised historical group average. Modelled EBITDA equals approximately 11.1% of EUR 184.6 million revenue. That margin allows for medical-service costs and price competition while recognising the value of contract recurrence and cross-referrals.

The assumption breaks through either membership loss or unit-margin compression. At 260,000 members and unchanged EUR 650 ARPU, revenue would be EUR 169 million; if EBITDA margin also fell to 9%, EBITDA would be only EUR 15.2 million and EV at 7.5x would be EUR 114 million, EUR 39 million below base. Conversely, 300,000 members at EUR 680 ARPU and a 12% margin would produce EUR 24.5 million EBITDA and EUR 184 million EV. The range demonstrates that reported member count alone is insufficient: retention, realised ARPU and service cost must move together.

FINNISH OCCUPATIONAL-HEALTH COMPETITIVE COVERAGE

PROVIDER	COVERED INDIVIDUALS	INDICATIVE SHARE	EVIDENCE / LIMITATION
Pihlajalinna	284,000 (H1 2026)	Est. 13.5%	Revenue not separately reported
Terveystalo	722,000 (2025)	34.4%	25,000+ corporate customers
Mehiläinen	Not disclosed	Not available	Top recommendation ranking
Aava	Not disclosed	Not available	EUR 140.1m total rev. est.

08.3

Public-Sector Outsourcing Services

Business division deep dive — Profit engine.

THE PRIZE IS PUBLIC HEALTH AND SOCIAL-CARE EXPENDITURE

Public-Sector Outsourcing Services competes for portions of Finland's EUR 24.6 billion of annual health and social-welfare net operating expenditure in 2024, reported by THL in July 2026. Central-government funding for wellbeing services counties is approximately EUR 27.1 billion in 2026, a related but differently defined funding measure. The private-provider market is much smaller: Pihlajalinna defines its core addressable market at EUR 3.5 billion, while Terveystalo uses EUR 4.5 billion. The valuation uses the actual signed-contract base rather than assuming a fixed share of total public expenditure, because political willingness and procurement design—not theoretical need—determine what can be outsourced.

Pihlajalinna generated EUR 199.1 million of external Outsourcing Services revenue in FY2025 under the restated reporting structure. That equals less than 1% of the EUR 24.6 billion displaced-spend pool, yet the relevant comprehensive-outsourcing niche is concentrated: the Finnish Competition and Consumer Authority estimated that Mehiläinen, Terveystalo and Pihlajalinna together held roughly 80–90% of outsourcing and purchased health services in 2024. Precise individual shares are unavailable. The opportunity is therefore substantial but lumpy, with one procurement award or expiry capable of moving group revenue by double-digit percentages.

CONTRACT TRANSITIONS EXPLAIN THE REVENUE RESET

The Kuusiolinna Terveys agreement, with annual turnover of approximately EUR 100 million, ended on 31 December 2025. Jämsän Terveys expired in August 2025 and reduced 2025 revenue by approximately EUR 19 million versus 2024, while Kolmostien Terveys was extended only through 30 April 2026 for transition. Earlier company material estimated that agreements ending in 2025 could reduce 2026 revenue by EUR 115 million. The latest 22 July 2026 guidance instead quantified the combined impact of expiries and residential-care divestments at approximately EUR 85 million. This report weights the newer EUR 85 million estimate because it incorporates transition timing and subsequent portfolio developments.

PUBLIC-SECTOR
OUTSOURCING SERVICES FACT
SHEET

PROFIT ENGINE

MODEL ESTIMATE REVENUE
EUR 199m

MODEL ESTIMATE COMPARABLE EBIT
EUR 27m

FY2025 EXTERNAL REVENUE
EUR 199.1m

FY2025 EBIT MARGIN
13.4%

2026 EXPIRY IMPACT
About EUR 85m

NORTHERN PIIRKANMAA VALUE
At least EUR 222m

CONTRACT FLOOR
Through 31 July 2031

REVENUE PER RESIDENT
Est. EUR 2,284/year

2027 VALUATION METRIC
EUR 32.0m EBITDA

SELECTED EV/EBITDA
4.0x

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.3

Public-Sector Outsourcing Services (continued)

Business division deep dive — Profit engine.

Northern Pirkanmaa provides the principal replacement. The contract has a minimum estimated value of EUR 222 million, covers about 18,000 residents and may terminate no earlier than 31 July 2031. Dividing EUR 222 million by 5.4 years and 18,000 residents gives this report's estimate of EUR 2,284 annual revenue per resident. That is around 52% of THL's EUR 4,395 annual health and social-welfare operating cost per inhabitant, consistent with the contract covering a defined service scope rather than every public-care cost. An expansion option for Mänttä-Vilppula and Juupajoki could add approximately EUR 30 million of annual revenue, but the base valuation assigns no value until exercised.

- Pihlajalinna Outsourcing Services: EUR 199.1m external revenue and EUR 26.7m EBIT in FY2025.
- Northern Pirkanmaa: at least EUR 222m total contract value for roughly 18,000 residents through July 2031.
- Terveystalo Portfolio Businesses: EUR 192.5m revenue in 2025, but the segment includes activities beyond comprehensive outsourcing.
- Top-three private chains: approximately 80–90% aggregate share of outsourcing and purchased health services in 2024.

UNIT ECONOMICS DEPEND ON RISK ALLOCATION, NOT JUST HEADLINE MARGIN

FY2025 Outsourcing Services EBIT was EUR 26.7 million, equal to 13.4% of EUR 199.1 million external revenue. That margin should not be extrapolated mechanically because contract mix changes after the large expiries. Kuusiolinna left demanding specialised-care cost liability with Pihlajalinna through termination; contracts with narrower risk transfer may have lower revenue but more predictable margins. Conversely, comprehensive contracts can generate attractive returns when utilisation, referral patterns and purchasing are managed below indexed contract revenue. The absence of disclosed cost per resident prevents a precise normalisation.

Employee-benefit expense represented 66.8% of Outsourcing Services revenue in Q4 2025, making wage inflation especially consequential. A 2.22% wage rise applied to that ratio increases costs by approximately 1.48% of revenue before mitigation. On EUR 150 million of post-expiry revenue, the first-order annual burden would be about EUR 2.2 million. Contract indexation terms for Northern Pirkanmaa were not disclosed in the supplied evidence, so the valuation assumes indexation broadly covers sector wage inflation with a timing lag. Failure of that assumption would quickly compress the pool's double-digit historical margin.

WORKING CAPITAL REDUCES THE QUALITY OF ACCOUNTING PROFIT

The Northern Pirkanmaa transition and Healthcare Services tied up working capital in trade receivables because of longer payment terms during Q2 2026. This matters disproportionately given the locked EUR 273 million net-debt position. A contract can report EBIT while consuming cash if invoicing, acceptance or county payment cycles extend. The model forecasts only EUR 16 million of 2026 FCFF despite EUR 88 million EBITDA, reflecting capex and working-capital needs. Cash conversion should therefore be assessed at contract level rather than inferred from segment margin.

CORE ANALYSIS · BUSINESS DIVISION DEEP DIVES

08.3

Public-Sector Outsourcing Services (continued)

Business division deep dive — Profit engine.

The transition also required workforce adjustment. April 2026 negotiations resulted in a permanent reduction of 139 Outsourcing Services roles, while earlier group reorganisation removed 88 roles mainly from administration. These actions may protect margins after revenue loss, but restructuring costs and service-transition execution can obscure the steady-state result. The base case assumes the savings support 2027 EBITDA without degrading service quality or triggering contractual penalties. Rising complaints, temporary labour use or receivable days would be early warnings that the cost programme is not translating into cash.

PROCUREMENT CONCENTRATION IS BOTH A MOAT AND A STRUCTURAL RISK

Scale, clinical governance and the capacity to assume service risk limit the number of credible comprehensive-outsourcing bidders. That supports the aggregate 80–90% share held by the three large chains. Yet buyer concentration is even stronger: wellbeing services counties control procurement, contract interpretation and renewal. Financially pressured counties may outsource to stabilise operations, but they may also renegotiate prices, narrow scopes or insource politically sensitive services. Three counties entered Ministry of Finance evaluation procedures in 2025, illustrating fiscal stress without identifying a direct Pihlajalinna exposure in the supplied evidence.

Specialised healthcare and elderly services together account for 47% of county net operating costs per capita, making these the largest potentially procurable blocks. They are also clinically and politically complex, so market size cannot be translated directly into contract wins. The Northern Pirkanmaa award confirms that comprehensive arrangements remain possible after the wellbeing-county reform. The expiry history confirms that duration is finite even when operating performance is acceptable. Valuation therefore applies a 4.0x EBITDA multiple, materially below private-healthcare pools.

08.3

Public-Sector Outsourcing Services (continued)

Business division deep dive — Profit engine.

VALUATION RECOGNISES CASH FLOW AND RENEWAL RISK

The SOTP assigns Public-Sector Outsourcing Services EUR 128 million of EV, calculated as EUR 32 million of modelled 2027 EBITDA multiplied by 4.0x. This multiple is below Terveystalo's 5.2x 2027E group EV/EBITDA and below Nordic care peers because the pool has buyer concentration, expiry cliffs and working-capital volatility. The EUR 32 million EBITDA estimate is not equivalent to the FY2025 EUR 26.7 million EBIT: it includes depreciation and assumes cost actions partly offset the smaller revenue base. On modelled post-reset revenue of roughly EUR 145–160 million, it implies an EBITDA margin near 20%, requiring favourable depreciation intensity and contract profitability.

That is the most debatable operating input in the SOTP. At EUR 25 million EBITDA and 4.0x, EV would be EUR 100 million, removing EUR 28 million or EUR 1.25 per share. At EUR 35 million and 4.5x, EV would reach EUR 157.5 million, adding EUR 29.5 million. The downside is most likely if indexation lags wages while receivables remain elevated; the upside requires Northern Pirkanmaa to settle quickly, legacy liabilities to disappear and the expansion option to be exercised. No value is assigned to the EUR 30 million annual option in the base case.

WHAT WOULD BREAK THE POOL

The division's thesis breaks if the post-expiry contracts cannot produce cash returns commensurate with their accounting margin. Specific failure conditions are EBITDA below EUR 25 million, persistent working-capital outflow after transition, or a material adverse renegotiation of Northern Pirkanmaa before the July 2031 minimum termination point. A renewed cycle of comprehensive contract wins would disprove the low-multiple assumption, but it would also increase concentration and execution exposure. Investors should therefore value signed economics, not bid-pipeline headlines.

PUBLIC-SECTOR OUTSOURCING SCALE AND CONTRACT CONCENTRATION			
OPERATOR / CONTRACT	REVENUE OR VALUE	DURATION / SCALE	COMPARABILITY
Pihlajalinna outsourcing	EUR 199.1m (2025)	Multiple contracts	Pure reported segment
Northern Pirkanmaa	EUR 222m minimum	18,000 residents; to 2031	New complete outsourcing
Terveystalo portfolio	EUR 192.5m (2025)	Portfolio Businesses	Includes other activities
Kuusiolinna Terveys	EUR 100m/year	Ended Dec. 2025	Historic Pihlajalinna contract
Top three chains	80–90% aggregate share	Finland (2024)	Individual shares unavailable

CORE ANALYSIS · CROSS-DIVISION BRIDGE

Putting the divisions back together

09

How the business division valuations aggregate to the group, and the re-rating the target price requires.

Occupational contracts are a customer-acquisition channel for private specialists, diagnostics and procedures. Revenue is assigned to the service delivered, so referral benefits are reflected in utilisation rather than counted twice.

Private Medical Services supplies clinic infrastructure and clinician networks that can support occupational delivery. Shared scheduling, digital triage and administration should improve utilisation, but undisclosed corporate costs limit precise standalone-margin reconstruction.

Outsourcing generates scale and clinical employment but adds receivables, contract concentration and transition costs. Its cash conversion determines how quickly group net debt falls and therefore whether operating improvement reaches equity holders.

BRIDGE MECHANICS

Each pool is valued on modelled 2027 EBITDA at a risk-specific multiple. The parts sum to the locked EUR 512 million EV; the locked EUR 276 million combined debt and other-claim deduction creates EUR 236 million of equity.

At EUR 10.50, Pihlajalinna trades at 5.79x 2026E EBITDA and 13.10x EBIT on the authoritative model. Those ratios are not obviously distressed relative to Terveystalo's 5.5x and 8.7x respectively.

The reverse DCF reaches EUR 10.50 with 2026 EBITDA of EUR 81.82 million, falling to EUR 74.61 million in 2027 before recovering. Terminal EBIT margin is only 6.4%, so the market does not require the followed model's full near-term earnings.

Pihlajalinna's normalised historical EV/EBITDA is 8.14x, but applying it indiscriminately would ignore the expiry and working-capital risk in outsourcing. The selected pool multiples deliberately bracket that history.

The ROE/PBV scatter implies EUR 13.6, while the SOTP implies EUR 10.5. The gap is a useful upside map, not a target: achieving it requires forecast ROE without a leverage or governance discount.

CORE ANALYSIS · SCENARIOS & VERDICT

10

Scenario logic and the bottom-line judgment

How the conclusion changes under less favourable scenarios.

Bear assumes 2027 revenue of EUR 560 million and EBITDA of EUR 70 million, a 12.5% margin. Private volumes soften after reimbursement cuts, occupational repricing lags wages and VAT, and outsourcing transition costs persist. The weaker cash result prevents meaningful deleveraging.

Base assumes EUR 600 million of revenue and EUR 87 million of EBITDA, a 14.5% margin. Healthcare pricing offsets wage inflation, occupational membership remains near 284,000 and Northern Pirkanmaa operates without material adverse renegotiation. Net debt follows the model toward EUR 249 million.

Bull assumes EUR 625 million revenue and EUR 105 million EBITDA, a 16.8% margin. Private utilisation improves, occupational ARPU rises without churn, and outsourcing achieves strong indexation and working-capital normalisation. Higher free cash flow permits faster debt reduction, so the scenario does not carry base-case net debt unchanged.

The scenario spread is driven more by EBITDA and cash conversion than revenue alone. The equity is levered to both because EUR 273 million of net debt exceeds market capitalisation.

BOTTOM LINE

Pihlajalinna offers identifiable operating upside, but the current EUR 10.50 price already corresponds to a defensible combination of healthcare resilience, outsourcing normalisation and continued leverage. HOLD is appropriate until contract-level cash conversion or Healthcare Services growth provides evidence that the group deserves a higher multiple rather than merely a cyclical earnings rebound.

FINANCIAL BRIDGE · PART 1

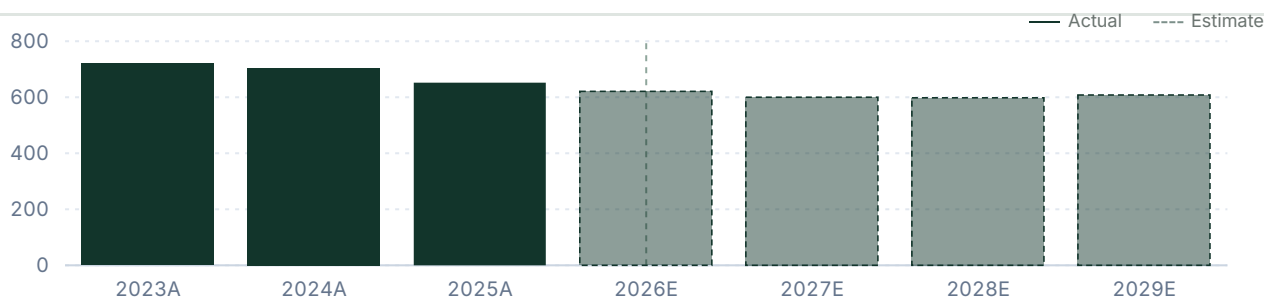
Net sales, EBITDA / EBIT, EBIT margin

14

Group-level trajectory for sales, earnings and margin; shaded bands mark forecast periods.

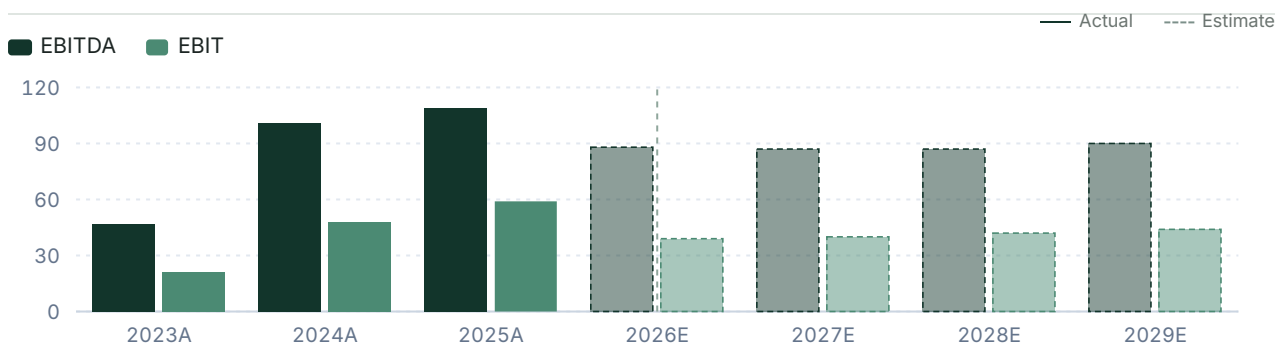
Net sales

EUR millions



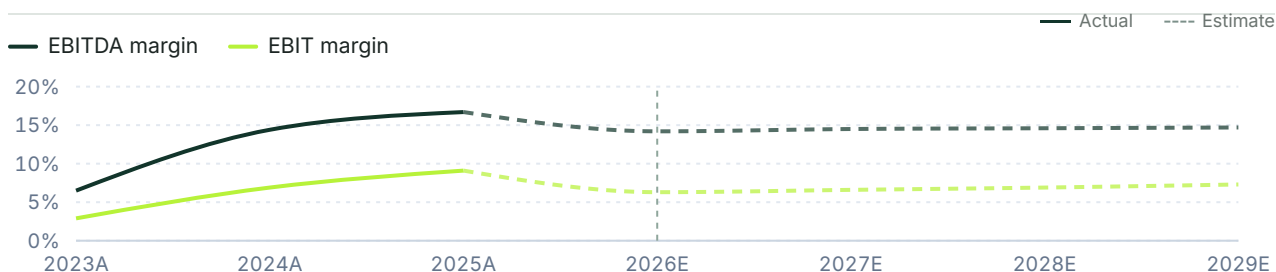
EBITDA & EBIT

EUR millions



EBIT and EBITDA margin

% of net sales



FINANCIAL BRIDGE · PART 2

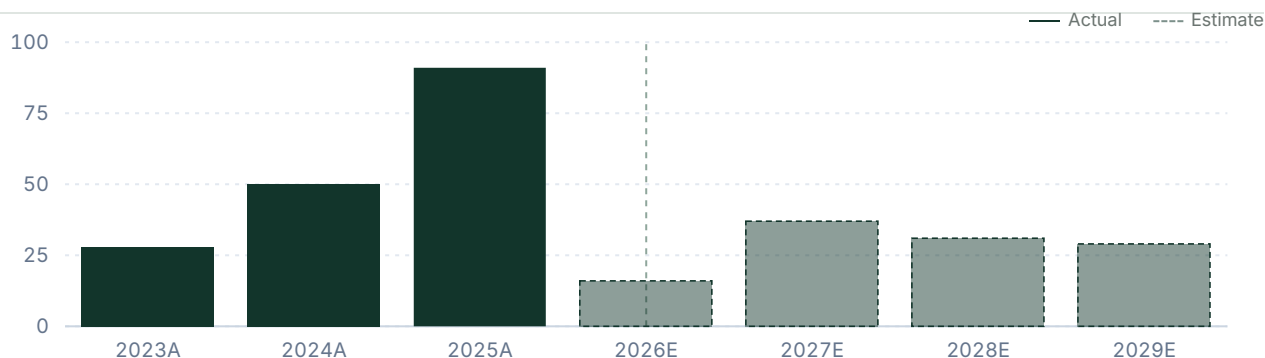
15

Free cash flow, leverage, and key financials

Cash generation and balance-sheet capacity.

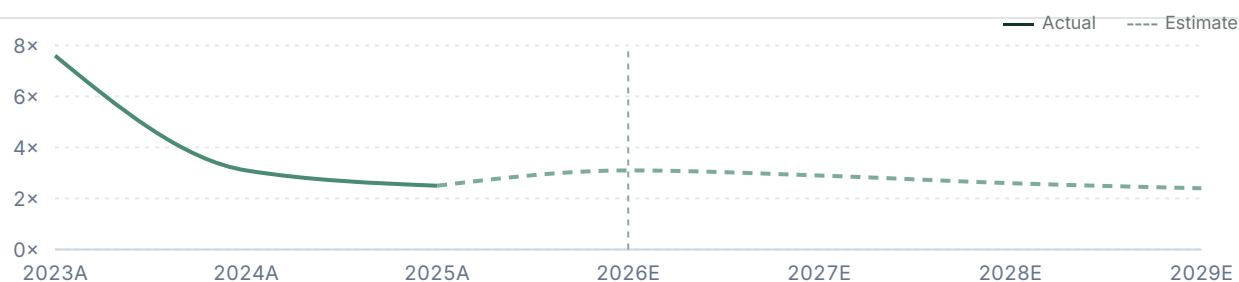
Free cash flow

EUR millions



Net debt / EBITDA

Ratio



KEY FINANCIALS

	2025 A	2026 E	2027 E	2028 E	2029 E
Net Sales	652	621	600	598	608
EBITDA	109	88	87	87	90
Comparable EBIT	59	39	40	42	44
Net Earnings	39	19	20	22	24
EPS (EUR)	1.7	0.9	0.9	1	1.1
DPS (EUR)	0.4	0.1	0.1	0.2	0.2

E = Valuatium estimate (not yet actualised)

VALUATION

Target price derivation

16

The rationale for the chosen valuation method and multiple, with the company's historical multiples.

SOTP is primary because private medical, occupational and public-outsourcing earnings have different recurrence, buyer concentration and cash-conversion risk. We apply 6.6x, 7.5x and 4.0x to modelled 2027 EBITDA, respectively. These multiples sit around direct-peer forward levels but below Pihlajalinna's 8.14x normalised history where contract risk warrants it. The parts carry 100% weight and sum to the locked EUR 512 million EV. Sensitivity shows that leverage makes even moderate EBITDA or multiple changes material to equity.

What the market is pricing.

The current EUR 236m equity value plus EUR 276m of net debt and other claims implies EUR 512m EV. Our private-medical and occupational values total EUR 384m, leaving EUR 128m for outsourcing. At 4.0x, the market therefore prices approximately EUR 32m of sustainable outsourcing EBITDA if the two healthcare pools earn EUR 55.4m.

The market assumes The July guidance cut proves permanent group demand erosion. We assign a different reading: Most of the revenue reset is tied to identified outsourcing expiries and divestments; healthcare franchise value should be assessed separately, although no valuation upside is awarded without delivery. (Pihlajalinna, financial targets and profit warning, 22 July 2026)

The market assumes FY2025 outsourcing margin can be capitalised at a healthcare multiple. We assign a different reading: Contract concentration, wage exposure and receivables justify only 4.0x 2027E EBITDA, preventing an inflated target despite strong FY2025 EBIT. (Pihlajalinna segment information, 31 March 2026; H1 report, 23 July 2026)

HISTORICAL AND FORECAST MULTIPLES								
METRIC	2020A	2021A	2022A	2023A	2024A	2025A	2026E	NORM. AVG. 2020-2025 (OUTLIERS DROPPED)
P/E	23.70x	14.96x	25.09x	34.62x	7.84x	8.52x	12.25x	19.12x
P/E adj.	24x	15x	25x	35x	8x	9x	12x	19x
EV/EBITDA	8.13x	7.92x	10.93x	10.91x	5.42x	5.53x	5.79x	8.14x
EV/EBIT	22.91x	17.49x	71.03x	24.69x	11.34x	10.20x	13.10x	17.33x
EV/Sales	—	0.85x	0.83x	0.71x	0.78x	0.93x	0.82x	0.82x
P/FCF	-230.25x	19.70x	-1.08x	5.78x	4.69x	3.61x	14.71x	4.69x
P/BV	1.93x	2.40x	1.55x	1.24x	1.38x	1.93x	1.30x	1.74x
P/S	—	0.49x	0.28x	0.22x	0.34x	0.50x	0.38x	0.37x
Dividend yield	—	1.6%	3.5%	—	0.7%	2.6%	1.3%	2.1%

VALUATION

Peers and target price bridge

17

Validated peer multiples, each method's implied price and weight, and the sensitivity of the target.

PEER COMPARISON				
COMPANY	PEER TYPE	2026E EV/EBITDA	2027E EV/EBITDA	2026E EBIT MARGIN
Pihlajalinna	Subject	5.79x	5.89x	6.3%
Terveystalo	Direct / high	5.5x	5.2x	12.3%
Attendo	Adjacent / medium	8.1x	7.6x	9.4%
Ambea	Adjacent / medium	5.5x	7.1x	9.3%
Humana	Adjacent / medium	5.2x	3.6x	2.2%
Peer median	Resolved peers	5.5x	6.2x	9.4%

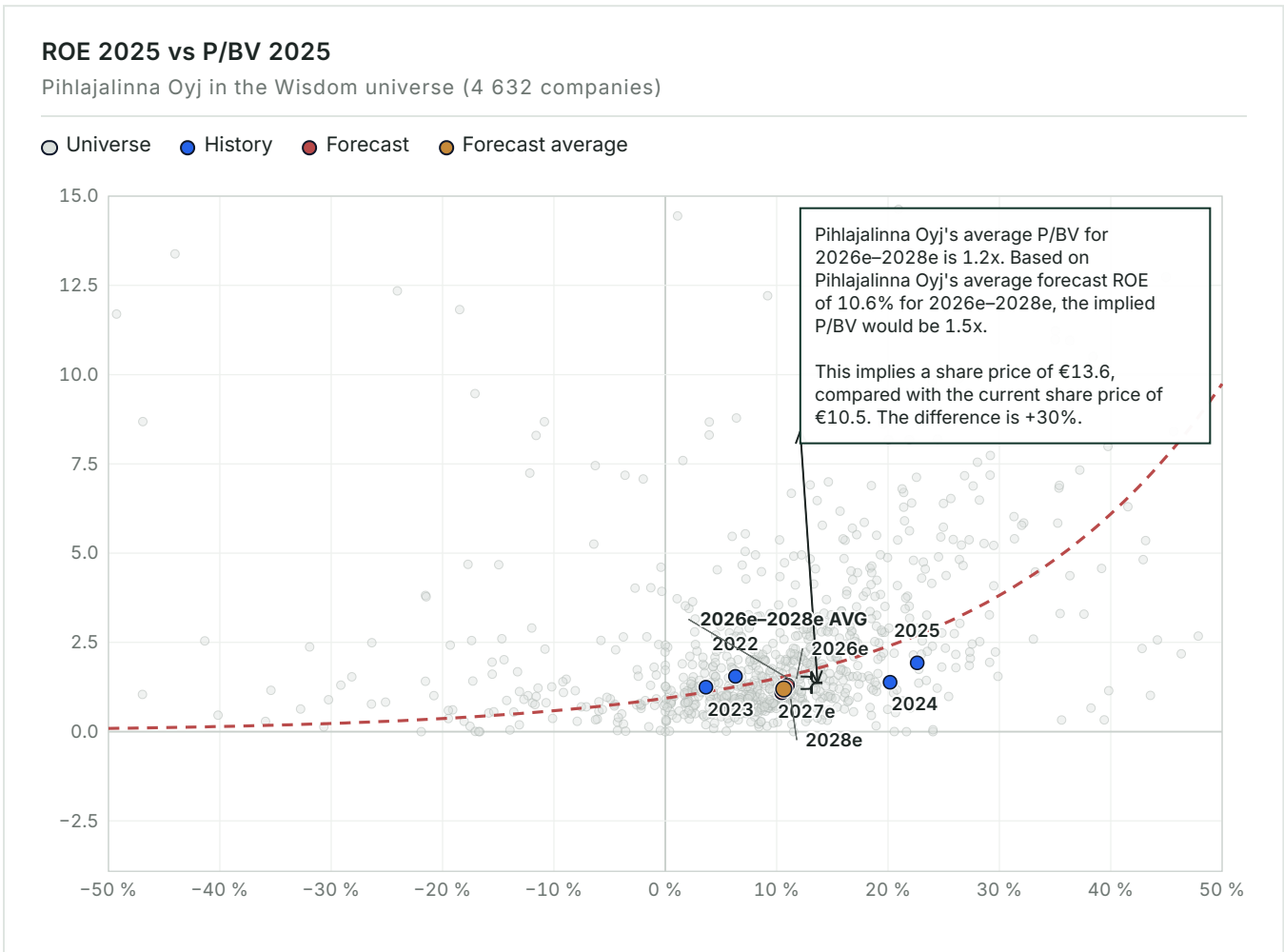
TARGET PRICE BRIDGE					
METHOD	FORECAST METRIC	METRIC VALUE	SELECTED MULTIPLE	IMPLIED PRICE (12M DISC.)	WEIGHT
· Private Medical Services	2027E Modelled EBITDA	35	6.6x	10.3 EUR	
· Occupational Healthcare	2027E Modelled EBITDA	20.4	7.5x	6.8 EUR	
· Public - Sector Outsourcing Services	2027E Modelled EBITDA	32	4.0x	5.7 EUR	
Sum of the parts	2027E Aggregate modell...	n/a	n/a	10.5 EUR	100%
Weighted target price				10.5 EUR	100%

VALUATION MAP

ROE vs P/BV — pricing versus profitability

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The whole Wisdom universe as context: how the market prices profitability on average, and what the company's forecast position would imply for the share price.



Pihlajalinna's average 2026–2028 forecast ROE of 10.6% and P/BV of 1.2x sit below the 1.5x P/BV implied by the 4,632-company universe curve. Applying 1.5x to average forecast book value of EUR 8.84 gives EUR 13.6 per share, 30% above EUR 10.5. The signal supports upside only if forecast ROE is delivered; leverage and contract concentration justify a discount to a broad cross-sector relationship.

MULTIPLES & SENSITIVITY

19

Forward multiples and EBITDA × multiple grid

How the implied valuation responds to changes in EBITDA and the applied multiple.

FORWARD MULTIPLES	
	2026 <i>E</i>
P/E	12.2×
EV/EBITDA	5.8×
EV/EBIT	13.1×
P / Valuatum FOCF	14.7×
P/BV	1.3×
Dividend Yield	1.3%
Net Debt / EBITDA	3.1×

E = Valuatum estimate (not yet actualised)

EBITDA × EV/EBITDA sensitivity						
2027E EBITDA (EURM) EV/EBITDA →	4.9×	5.4×	5.9×	6.4×	6.9×	
-20% EUR 71m	EUR 348m EUR 3.20 / sh	EUR 383m EUR 4.80 / sh	EUR 419m EUR 6.40 / sh	EUR 454m EUR 7.90 / sh	EUR 490m EUR 9.50 / sh	
-10% EUR 80m	EUR 392m EUR 5.20 / sh	EUR 432m EUR 6.90 / sh	EUR 472m EUR 8.70 / sh	EUR 512m EUR 10.50 / sh	EUR 552m EUR 12.30 / sh	
Base EUR 88m	EUR 431m EUR 6.90 / sh	EUR 475m EUR 8.90 / sh	EUR 519m EUR 10.80 / sh	EUR 563m EUR 12.80 / sh	EUR 607m EUR 14.70 / sh	
+10% EUR 97m	EUR 475m EUR 8.90 / sh	EUR 524m EUR 11.00 / sh	EUR 572m EUR 13.20 / sh	EUR 621m EUR 15.30 / sh	EUR 669m EUR 17.50 / sh	
+20% EUR 106m	EUR 519m EUR 10.80 / sh	EUR 572m EUR 13.20 / sh	EUR 625m EUR 15.50 / sh	EUR 678m EUR 17.90 / sh	EUR 731m EUR 20.30 / sh	

SCENARIO VALUATION

20

Bear / Base / Bull bridge

Revenue, EBITDA, multiple, EV, equity, value-per-share and upside in each scenario.

SCENARIO BRIDGE

	REVENUE	EBITDA	MARGIN	MULTIPLE	EV	EQUITY	VALUE / SHARE	UPSIDE
Bear	560	70	12.5%	4.8×	336	60	EUR 2.67	-74.5%
Base	600	87	14.5%	5.9×	513	237	EUR 10.56	+0.5%
Bull	625	105	16.8%	6.7×	704	428	EUR 19.01	+81.1%

KEY CONDITIONS

Bear requires simultaneous healthcare volume weakness, incomplete occupational repricing and outsourcing cash absorption. Base requires stable covered lives, price recovery and ordinary Northern Pirkanmaa execution. Bull requires higher utilisation, successful contract indexation and enough free cash flow to reduce net debt materially below the base path. These scenarios are operating boundary cases, not probability-weighted target-price inputs. The code-verified 12-month target of EUR 10.5 sits between Bear and Base (Bear EUR 2.7, Base EUR 10.6, Bull EUR 19.0); its method weights are shown in the Target Price Bridge.

THESIS BREAKER

EBITDA below EUR 75 million combined with net debt above EUR 270 million after 2027 invalidates the base valuation.

CATALYSTS

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Monitoring checklist

Near-, medium- and long-term events that could change the recommendation.

CATALYST REGISTER				
	TIMING	SEGMENT	INDICATOR	VALUATION IMPACT
Q3 report tests revised 2026 guidance	NEAR-TERM	Public-Sector Outsourcing Services	Revenue and adjusted EBITA versus EUR 550–570m guidance	Confirms or cuts transition EBITDA
Evidence of private-volume resilience after Kela cuts	NEAR-TERM	Private Medical Services	Organic revenue, visits or clinician utilisation	Supports 6.6x pool multiple
Occupational pricing absorbs VAT reclassification	NEAR-TERM	Occupational Healthcare	Membership retention and realised price	Protects EUR 20.4m EBITDA
Northern Pirkanmaa working capital normalises	MEDIUM-TERM	Public-Sector Outsourcing Services	Receivables and operating cash flow	Reduces leverage discount
2027 wage increases passed through	MEDIUM-TERM	Occupational Healthcare	Revenue per member versus personnel cost	Enables margin recovery
Net debt approaches EUR 249 million	MEDIUM-TERM	Private Medical Services	Net debt and FCFF	Adds equity value directly
Northern Pirkanmaa expansion option exercised	LONG-TERM	Public-Sector Outsourcing Services	EUR 30m annual revenue option	Upside not in base value
READ - THROUGH Medium-term upside requires cash conversion. EBITDA improvement that remains trapped in receivables will not reduce the leverage discount, while a visible path toward EUR 249 million of net debt would transfer operating value to equity. The Northern Pirkanmaa expansion option is genuine long-term upside but is excluded from the base valuation until exercised.				

RISKS

Risk register

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Each risk's business division, mechanism, early-warning trigger, impact band and structural type.

RISK REGISTER					
	SEGMENT	MECHANISM	EARLY WARNING	IMPACT	TYPE
Private-care volume falls after reimbursement cuts	Private Medical Services	Higher patient out-of-pocket cost reduces visits and clinic utilisation.	Organic private revenue or disclosed visits decline despite price increases.	HIGH	MANAGEABLE
Occupational contracts fail to recover VAT	Occupational Healthcare	Tax reclassification is absorbed in fixed-price contracts or causes usage reductions.	Revenue per member stagnates while membership or margin declines.	HIGH	MANAGEABLE
Wage inflation exceeds pricing	Occupational Healthcare	Scheduled pay increases and scarce specialists raise unit costs faster than realised prices.	Personnel-cost ratio rises after September 2026 price actions.	HIGH	THESIS BREAKER
Outsourcing agreements expire or are renegotiated	Public-Sector Outsourcing Services	Wellbeing counties reduce scope, insource services or impose weaker indexation.	Procurement notices, adverse county decisions or reduced contract guidance.	HIGH	STRUCTURAL
Receivables absorb outsourcing cash flow	Public-Sector Outsourcing Services	Longer payment terms and transition invoicing keep accounting profit from reducing debt.	Operating cash flow trails EBITDA and receivables continue increasing.	HIGH	THESIS BREAKER
Leverage breaches financial flexibility	Private Medical Services	Weak EBITDA or cash conversion limits covenant headroom before June 2028 refinancing.	Net debt/EBITDA approaches the 3.75x covenant or gearing remains elevated.	HIGH	THESIS BREAKER
Clinical capacity and quality deteriorate	Occupational Healthcare	Cost reductions or practitioner scarcity weaken access, retention and referral conversion.	Longer booking times, rising temporary labour or customer attrition.	MEDIUM	MANAGEABLE
		Lower long-term			

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APPENDIX

Income statement

INCOME STATEMENT — EUR MILLIONS						
	2023 A	2024 A	2025 A	2026 E	2027 E	2028 E
Net Sales	720	704	652	621	600	598
EBITDA	47	101	109	88	87	87
EBITDA margin	6.5%	14.4%	16.7%	14.2%	14.5%	14.6%
Depreciation	-26	-53	-50	-49	-47	-46
Operating Profit (EBIT)	21	48	59	39	40	42
EBIT margin	2.9%	6.9%	9.1%	6.3%	6.6%	6.9%
Net financial items	-12	-10	-13	-13	-13	-12
Pre-tax Profit	8	39	46	26	27	29
Net Earnings	5	30	39	19	20	22
PER SHARE						
EPS	0.2	1.3	1.7	0.9	0.9	1
DPS	0	0.1	0.4	0.1	0.1	0.2
Payout ratio	—	5.2%	22.2%	15.4%	15.4%	15.4%

E = Valuatum estimate (not yet actualised)

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APPENDIX

Balance sheet

BALANCE SHEET — EUR MILLIONS						
	2023 A	2024 A	2025 A	2026 E	2027 E	2028 E
ASSETS						
Tangible assets	270	249	227	217	210	209
Intangibles	21	16	15	14	14	14
Goodwill	252	255	254	254	254	254
Non-current assets	550	525	499	488	480	480
Inventories	4	5	4	4	4	4
Receivables	63	62	56	54	52	52
Cash & equivalents	25	31	31	29	28	27
Current assets	92	97	91	86	83	83
Total Assets	657	630	598	582	572	571
EQUITY & LIABILITIES						
Equity	125	169	173	183	200	219
Long-term debt	144	133	103	57	47	38
Short-term debt	3	2	2	57	47	38
Long-term liabilities	115	87	95	49	40	30
Current liabilities	159	155	126	154	143	133
Total liabilities & equity	637	630	598	582	572	571
CAPITAL STRUCTURE & RATIOS						
Net debt	353	316	271	273	249	229
Capital invested	247	273	246	269	267	267
Equity ratio	19.5%	26.8%	28.9%	31.5%	35.0%	38.4%
Gearing	283.4%	187.1%	157.1%	149.1%	124.2%	104.8%
Net debt / EBITDA	7.6×	3.1×	2.5×	3.1×	2.9×	2.6×
Current ratio	0.6	0.6	0.7	0.6	0.6	0.6

E = Valuatum estimate (not yet actualised)

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APPENDIX

Cash flow

CASH FLOW — EUR MILLIONS						
	2023 A	2024 A	2025 A	2026 E	2027 E	2028 E
OPERATIONS						
Cash from operations (model)	51	81	66	45	67	67
Operating cash flow (Valuatum calculation)	64	95	76	55	76	76
Change in working capital	-18	2	23	23	0	0
INVESTING & FREE CASH						
Gross capex	34	28	24	39	39	45
Capex (ex. M&A)	-34	-28	-24	-39	-39	-45
Cash after capex (CFO - gross capex)	17	54	42	6	28	22
Free operating cash flow (Valuatum def.)	28	50	91	16	37	31
Free cash flow to firm	28	50	91	16	37	31
FINANCING						
CF from financing	-26	-34	-41	-9	-28	-23
Dividends paid	-7	—	-2	-9	-3	-3
Net change in cash	-9	20	1	-2	-1	-0

E = Valuatum estimate (not yet actualised)

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APPENDIX

Quarterly snapshot

HISTORICAL QUARTERS — LAST TWO COMPLETED YEARS, EUR MILLIONS

	Q1/24	Q2/24	Q3/24	Q4/24	Q1/25	Q2/25	Q3/25	Q4/25
Net Sales	183	175	164	183	181	171	149	151
EBITDA	25	23	26	27	29	25	26	23
EBITDA margin	13.6%	12.9%	16.0%	14.8%	16.0%	14.8%	17.6%	15.4%
Comparable EBIT	12	10	12	15	16	13	13	11
EBIT margin	6.6%	5.4%	7.1%	8.0%	9.0%	7.6%	8.9%	7.4%
Net Earnings	8	6	7	10	12	9	12	6
EPS	0.4	0.2	0.3	0.4	0.5	0.4	0.5	0.3

CURRENT YEAR — QUARTERLY, EUR MILLIONS

	Q1/26	Q2/26	Q3/26 <i>E</i>	Q4/26 <i>E</i>
Net Sales	142	136	132	144
EBITDA	24	16	25	23
EBITDA margin	17.1%	11.6%	18.6%	15.8%
Comparable EBIT	12	4	12	11
EBIT margin	8.4%	2.6%	8.9%	7.4%
Net Earnings	9	0	9	5
EPS	0.4	0	0.4	0.2

E = Valuatum estimate (not yet actualised)

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Research sources

Direct links for the evidence used in the report.

SOURCES REGISTER — 1 OF 10			
	VALUE	CATEGORY	USED IN
Financial statements and forecast model	Valuatum Wisdom model 11074, snapshot period 2026	FINANCIAL SNAPSHOT	Financials, forecasts & valuation
Share-price history and market profile	Financial Modeling Prep, PIHLIS.HE financialmodelingprep.com	MARKET DATA	Price chart & market facts
Pihlajalinna revises its financial reporting structure and...	www.inderes.fi	OFFICIAL SOURCE	Reported segment table
Pihlajalinna estimates its core addressable market in Finla...	www.pihlajalinna.fi/.../addressable-market	OFFICIAL SOURCE	Private Medical Services deep dive
Pihlajalinna Oyj re-reported revenue of EUR 465.2 million for...	www.pihlajalinna.fi/.../reports-and-presentations	OFFICIAL SOURCE	Private Medical Services deep dive
Pihlajalinna's total revenue for the 2025 financial year re...	www.pihlajalinna.fi/.../investors	OFFICIAL SOURCE	Occupational Healthcare deep dive
Pihlajalinna's revenue is expected to decline in 2026 to be...	www.pihlajalinna.fi/.../financial-targets	OFFICIAL SOURCE	Occupational Healthcare deep dive
Pihlajalinna's hourly rate for an occupational health docto...	www.pihlajalinna.fi/.../prices	OFFICIAL SOURCE	Occupational Healthcare deep dive
The Wellbeing Services County of Central Finland terminated...	www.pihlajalinna.fi/.../reports-and-presentations	OFFICIAL SOURCE	Public-Sector Outsourcing Services deep dive
Pihlajalinna was selected for a new outsourcing arrangement...	www.pihlajalinna.fi/.../releases	OFFICIAL SOURCE	Public-Sector Outsourcing Services deep dive

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Research sources (continued)

Direct links for the evidence used in the report.

SOURCES REGISTER — 2 OF 10			
	VALUE	CATEGORY	USED IN
Pihlajalinna's adjusted EBITA margin for 2025 reached 10.0%...	www.pihlajalinna.fi/.../financial-statements	OFFICIAL SOURCE	Public-Sector Outsourcing Services deep dive
The service agreement for Kolmostien Terveys in Pirkanmaa i...	www.pihlajalinna.fi/.../financial-reports	OFFICIAL SOURCE	Public-Sector Outsourcing Services deep dive
Pihlajalinna adjusted EBITDA for the full year 2025 was EUR...	investors.pihlajalinna.fi	OFFICIAL SOURCE	Research evidence
Pihlajalinna reported a net working capital change of EUR - ...	investors.pihlajalinna.fi/.../q3-2025-en.pdf	OFFICIAL SOURCE	Research evidence
Pihlajalinna lowered its full-year 2026 revenue guidance to...	www.pihlajalinna.fi/.../financial-targets	MARKET DATA	Catalysts and risks
The company published its Half Year Financial Report for Ja...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
Pihlajalinna's pricing was updated to reflect a retroactive...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
Pihlajalinna distributed a dividend of EUR 0.53 per share f...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
Pihlajalinna revised its financial reporting structure into...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
A new operating model and management team, including CEO Tu...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks

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Research sources (continued)

Direct links for the evidence used in the report.

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	VALUE	CATEGORY	USED IN
Pihlajalinna announced the planned publication of its Inter...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
Pihlajalinna published its Financial Statements Release for...	www.pihlajalinna.fi	MARKET DATA	Catalysts and risks
Terveystalo is the market leader in Finnish occupational he...	www.terveystalo.com/.../terveystalos-addressable-markets	MARKET DATA	Occupational Healthcare deep dive
Pihlajalinna's share of the Finnish working population rece...	www.ncbi.nlm.nih.gov/.../PMC11011883	MARKET DATA	Occupational Healthcare deep dive
The core addressable market for private healthcare in Finla...	www.pihlajalinna.fi/.../addressable-market	MARKET DATA	Occupational Healthcare deep dive
Mehiläinen is the largest private health-care provider in Fi...	www.mehilainen.fi/.../results-and-key-figures	MARKET DATA	Public-Sector Outsourcing Services deep dive
Terveystalo's market share of the entire privately provided...	www.terveystalo.com/.../terveystalos-addressable-markets	MARKET DATA	Public-Sector Outsourcing Services deep dive
Pihlajalinna's Public Services segment, which produces soci...	www.inderes.fi	MARKET DATA	Public-Sector Outsourcing Services deep dive
Pihlajalinna's Private Healthcare Services segment reported...	storage.googleapis.com/.../annual-report-2025-pihlajalinna.pdf	MARKET DATA	Private Medical Services deep dive
Pihlajalinna employed approximately 4,500 employees and wor...	www.mfn.se/.../pihlajalinna-has-published-the-2025-a...	MARKET DATA	Private Medical Services deep dive

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Research sources (continued)

Direct links for the evidence used in the report.

SOURCES REGISTER — 4 OF 10			
	VALUE	CATEGORY	USED IN
Main competitor Terveystalo reported 7.2 million customer v...	www.terveystalo.com/.../company	MARKET DATA	Private Medical Services deep dive
Mehiläinen Group's total revenue for 2025 reached EUR 2,413...	www.mehilainen.fi	MARKET DATA	Private Medical Services deep dive
Pihlajalinna's occupational healthcare individual customer...	cdn.prod.nntech.io/.../presentation.pdf	MARKET DATA	Private Medical Services deep dive
The Finnish occupational healthcare system covers approxima...	www.pihlajalinna.fi/.../addressable-market	MARKET DATA	Occupational Healthcare deep dive
Pihlajalinna's occupational healthcare individual customer...	www.ncbi.nlm.nih.gov/.../PMC11222333	MARKET DATA	Occupational Healthcare deep dive
Pihlajalinna's Public Services segment reported revenue and...	pihlajalinna.fi/.../investors	MARKET DATA	Public-Sector Outsourcing Services deep dive
Terveystalo's Portfolio Businesses segment, which includes...	www.terveystalo.com/.../investors	MARKET DATA	Public-Sector Outsourcing Services deep dive
Mehiläinen's total group revenue for 2025, a major competit...	www.mehilainen.fi/.../investors	MARKET DATA	Public-Sector Outsourcing Services deep dive
Pihlajalinna revised its financial reporting structure as o...	investors.pihlajalinna.fi/.../stock-exchange-release-31-03-2026	MARKET DATA	Research evidence
The consolidated revenue for Pihlajalinna Group for the ful...	investors.pihlajalinna.fi/.../annual-report-2025	MARKET DATA	Research evidence

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Research sources (continued)

Direct links for the evidence used in the report.

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The complete social and healthcare out-sourcing agreement of...	news.cision.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
Pihlajalinna was selected for a new complete outsourcing co...	news.cision.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
The Northern Pirkanmaa contract includes an expansion optio...	news.cision.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
The Jämsän Terveys Oy service agreement with the Wellbeing...	news.cision.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
Complete outsourcing agreements ending at the end of 2025 w...	investors.pihlajalinna.fi/.../capital-markets-day-presentation-2025...	MARKET DATA	Public-Sector Outsourcing Services deep dive
Liability for demanding specialized care costs remained wit...	news.cision.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
Change negotiations in April 2026 for the Northern Pirkanma...	www.nasdaq.com	MARKET DATA	Public-Sector Outsourcing Services deep dive
Pihlajalinna expects 2026 revenue to decline due to the exp...	investors.pihlajalinna.fi/.../financial-targets	MARKET DATA	Private Medical Services deep dive
Pihlajalinna issued a profit warning estimating 2025 revenu...	news.cision.com	MARKET DATA	Private Medical Services deep dive

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Research sources (continued)

Direct links for the evidence used in the report.

SOURCES REGISTER — 6 OF 10			
	VALUE	CATEGORY	USED IN
In Pihlajalinna's Outsourcing Services division, employee b...	www.inderes.fi	MARKET DATA	Research evidence
Finnish wellbeing-ser- vices- county (WSC) net operating costs...	thl.fi/.../healthcare-and-social-welfare-finances	MARKET DATA	Public-Sector Outsourcing Services deep dive
The Wellbeing Services County of Pirkanmaa appointed Pihlaj...	www.inderes.fi	MARKET DATA	Catalysts and risks
Terveystalo is the largest private health- care service provi...	www.terveystalo.com/.../terveystalo-as-an-investment	MARKET DATA	Private Medical Services deep dive
Mehiläinen is described as Finland's largest healthcare pro...	www.mehilainen.fi/.../result-and-key-figures	MARKET DATA	Private Medical Services deep dive
Terveystalo holds a market share of approx- imately 25-30 per...	www.terveystalo.com/.../addressable-markets	MARKET DATA	Private Medical Services deep dive
Lääkärikeskus Aava ja Pikkujätti is a family- owned Finnish...	bolddata.nl/.../top-50-largest-hospitals-nordics	MARKET DATA	Private Medical Services deep dive
Aava ja Pikkujätti Oy's revenue was recorded as EUR 140.1 m...	nordicleads.com/.../top-healthcare-companies-finland	MARKET DATA	Private Medical Services deep dive
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Sickness-related ab- sences and lost pro- ductivity in Finland...	www.helsinkitimes.fi/.../23456-study-sickness-absences-cost-fi...	MARKET DATA	Occupational Healthcare deep dive

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Research sources (continued)

Direct links for the evidence used in the report.

SOURCES REGISTER — 7 OF 10			
	VALUE	CATEGORY	USED IN
Mehiläinen, Terveystalo, and Pihlajalinna collectively hold...	www.kkv.fi	MARKET DATA	Public-Sector Outsourcing Services deep dive
Total public expenditure on healthcare and social welfare i...	thl.fi/.../expenditures-and-revenues	MARKET DATA	Public-Sector Outsourcing Services deep dive
Terveystalo estimates its market share in the Finnish priva...	www.terveystalo.com/.../terveystalos-addressable-markets	MARKET DATA	Private Medical Services deep dive
The total net operating expenditure for healthcare and soci...	thl.fi/.../healthcare-and-social-welfare-finances	MARKET DATA	Private Medical Services deep dive
Kela reimbursements for private healthcare appointments wit...	valtioneuvosto.fi	MARKET DATA	Private Medical Services deep dive
Sickness absences cost the Finnish economy approximately EU...	www.helsinkitimes.fi/.../25432-study-sickness-absences-cost-fi...	MARKET DATA	Occupational Healthcare deep dive
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A basic statutory occupational healthcare package for small...	www.medfin.fi/.../statutory-occupational-healthcare	MARKET DATA	Occupational Healthcare deep dive
The Occupational Health Care Regulation in Finland is being...	www.terveystalo.com	MARKET DATA	Occupational Healthcare deep dive
On January 1, 2026, occupational safety and health authorit...	valtioneuvosto.fi	MARKET DATA	Occupational Healthcare deep dive

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Research sources (continued)

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Mehiläinen acquired the occupational health business of Hel...	www.mehilainen.fi	MARKET DATA	Occupational Healthcare deep dive
Total annual net operating costs for healthcare and social...	thl.fi/.../en	MARKET DATA	Public - Sector Outsourcing Services deep dive
For Q1 2026, Pihlajalinna reported an operating profit (EBI...	quartr.com/.../q1-2026	MARKET DATA	Research evidence
The Social Insurance Institution of Finland (Kela) will red...	valtioneuvosto.fi	MARKET DATA	Private Medical Services deep dive
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Reimbursement for private physiotherapy was reintroduced fo...	www.kela.fi	MARKET DATA	Private Medical Services deep dive
The annual healthcare payment ceiling (maksukatto) for publ...	suolistosairaudet.fi/.../sosiaalituran-muutokset-2026	MARKET DATA	Private Medical Services deep dive
Western Uusimaa Wellbeing Services County established the 2...	www.luvn.fi/.../palvelusetelit	MARKET DATA	Private Medical Services deep dive
Total central government funding for well-being services cou...	vm.fi	MARKET DATA	Private Medical Services deep dive
The Finnish private healthcare sector collective agreement...	www.superliitto.fi/.../terveyspalvelualan-tyoehtosopimus-tptes	MARKET DATA	Research evidence

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Direct links for the evidence used in the report.

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	VALUE	CATEGORY	USED IN
An additional specific pay-scale increase is mandated for d...	www.tehy.fi	MARKET DATA	Research evidence
Private healthcare employees will receive a local wage inst...	www.jhl.fi	MARKET DATA	Research evidence
The public SOTE-sopimus pay development program schedules a...	www.tehy.fi/.../sote-sopimus-2025-2028	MARKET DATA	Research evidence
The Finnish government reduced the minimum staffing require...	silkroad.fi/.../finnish-labor-market-situation	MARKET DATA	Research evidence
The total number of job vacancies in Finland was reported a...	tradingeconomics.com/.../job-vacancies	MARKET DATA	Research evidence
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Mehiläinen's core market in Finland accounted for 69 persen...	www.fitchratings.com	MARKET DATA	Private Medical Services deep dive
Lääkärikeskus Aava is a significant competitor with annual...	nordiclead.com/.../healthcare	MARKET DATA	Occupational Healthcare deep dive
Terveystalo's specific revenue from public-sector outsourci...	www.alphaspread.com/.../investor-relations	MARKET DATA	Public-Sector Outsourcing Services deep dive
Analyst consensus for Terveystalo's Portfolio Businesses se...	vara-services.com/.../terveystalo	MARKET DATA	Public-Sector Outsourcing Services deep dive

SOURCES & DISCLAIMER

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Sources, assumptions, and standard disclaimer

Direct research links, methodology assumptions, and the standard disclaimer.

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	VALUE	CATEGORY	USED IN
One lost working day due to sickness absence results in an...	www.ilmarinen.fi/.../calculate-the-cost-of-sick-leave	MARKET DATA	Occupational Healthcare deep dive
Pihlajalinna's active job postings in 2026 showed a signifi...	www.reveliolabs.com/.../pihlajalinna-employment-data-2026	MARKET DATA	Research evidence
The nurse shortage in Finland is projected to grow to over...	superkoulu.fi/.../nurse-demand-analysis-finland-2025-2030	MARKET DATA	Research evidence
The privately provided healthcare market in Finland is esti...	www.terveystalo.com/.../market-information	MARKET DATA	Public-Sector Outsourcing Services deep dive
The Ministry of Finance initiated evaluation procedures for...	eurohealthobservatory.who.int/.../analyses	MARKET DATA	Public-Sector Outsourcing Services deep dive

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